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EDITORIAL

Educational Innovation and Responsibility

THIS WHOLE ISSUE is devoted to the exploration of educational innovation in psychiatric mental health nursing. It meets 2 needs. First, it provides a useful collection of some model curricula that illustrate the range and diversity of experimentation that is currently afoot in the discipline. Second, it allows me to pilot a "special topic" issue of *Archives of Psychiatric Nursing*. I had been thinking about featuring an annual "special topic" issue when a flood of educational innovation manuscripts came across the transom. It seemed a good opportunity to float a trial balloon—so, here it is.

The focus on educational innovation was sparked by recent national discussions on the role, scope, and titling issues related to Advanced Practice Psychiatric Nursing, the conflict within the discipline surrounding those issues, and the announcement by the American Nurses Credentialing Center (ANCC) of the intention to sponsor a Psychiatric Nurse Practitioner exam. As the reader will soon discover in these pages, there is no unanimous opinion on the right direction for the discipline (not that there is necessarily only 1 right direction). What may not be so obvious is that this lack of unanimity is a good thing as long as we do not prematurely tinker with regulation and credentialing until the dust of innovation settles.

Herein lies the point of this editorial. It is too soon, way too soon, to be sponsoring a new exam for the discipline. There are too many very serious debates yet to be had and titling issues yet to be resolved. With all due respect, organizations like

the ANCC are in the *business* of credentialing and testing. However, their business is not to define the parameters and boundaries related to scope of practice that are to be tested—that is for the discipline to determine. When our credentialing organizations act too much like a business, the result is to provide a product based on "customer demand" whether the product is good for the profession or not and whether the demand is premature or not. Because programs have created new tracks should not be sufficient evidence to establish a new test. We already have tests, credentialing procedures, and titles for the Adult Nurse Practitioner (ANP), Pediatric Nurse Practitioner (PNP), Family Nurse Practitioner (FNP), and Clinical Nurse Specialist (CNS) roles and until we clarify how and if the Psychiatric Nurse Practitioner is different from these it is premature to create a new exam. Further, it is irresponsible for those of us in academic nursing to be making such demands of the ANCC before having a major national dialogue about the implications.

The challenge of educational innovation is to simultaneously equip the "pioneer" graduates of such programs with the prerequisite skills and knowledge to function in today's world while redefining the future of the discipline. In practical terms, this means we are obligated to ensure that our graduates can meet current standards for credentialing as a CNS even while equipping them with additional skills outside the usual boundaries of the CNS role. At the very least, we are obligated to tell applicants to our experimental programs what

obstacles they may face upon graduation, e.g., eligibility for certain titles and scope of practice in some states but not others, increased scrutiny by credentialing agencies and possible ineligibility for existing credentials, as well as the excitement and challenge of being clinical innovators. Frankly, many of our academic programs are also too much in the *business* mode. They are too ready to provide potential candidates whatever it takes to attract them to our programs and the discipline; yet, reluctant to take responsibility for the results of our educational innovations.

As the articles in this issue attest, there is a major need to examine our existing curricula and to bring the discipline rapidly to the brink of the new Millennium without bringing it to the brink of disaster. There is certainly a need for new content in the role. There is a case to be made for dual certification. All of these are possible without changing our current credentialing exams. I think

the jury is still out as to whether there is an utterly distinct, measurably different new role known as the Psychiatric Nurse Practitioner. Resolving these issues will take the kind of creativity exhibited in these pages, the capacity to tolerate chaos and the wisdom to resist premature change in regulation, titling, and credentialing until more of us are certain about the paths we should travel.

I hope to receive many Letters to the Editor on these matters in reaction not only to this editorial but to the models proposed herein. I also want you to weigh in on my proposal to feature one issue per year of *APN* devoted to a “special topic.” Do you like the idea? Why or why not? In your view, what topics warrant an entire issue? Let me hear from you!

Judith B. Krauss
Editor